

Verbal Order (Read Back) Policy

Introduction:

In the context of healthcare, a verbal order refers to a directive given by a healthcare provider to a member of the healthcare team regarding patient care, treatment, or medication, typically communicated orally. Verbal orders are often used in urgent or emergency situations when a written order may not be immediately feasible.

Scope:

This policy encompasses all health care professions dealing with patient care. This extends to doctors, nurses, pharmacists, respiratory therapists, dietitians, physical therapists, occupational therapists, speech therapists, and perfusionists.

Definition:

Verbal Orders are **emergent** or **urgent** face-to-face or telephone orders only.

Order means a direction given by a regulated health care professional to carry out specific activity (-ies) as part of the diagnostic and/or therapeutic care and treatment, to the benefit of a patient. An order may be written (including handwritten and or electronic), verbal (**e.g face to face, telephone**).

Indications:

The purposes of verbal orders in healthcare include:

1. Urgent Care: Providing immediate instructions for critical patient care when a written order is not immediately feasible.
2. Emergency Situations: Addressing emergent medical needs promptly and efficiently.
3. Flexible Care: Allowing for swift adjustments to patient care plans based on real-time clinical assessments.
4. Collaborative Communication: Facilitating prompt communication and decision-making within the healthcare team to ensure timely and appropriate care for patients.
5. Documentation: To establish clear guidelines for the use, verification, and documentation of verbal and telephone orders in healthcare settings.

Conditions:

1. In general, orders must be written electronically. Verbal and telephone orders should be used in emergency situations only.

2. Verbal orders may be given in emergencies. Emergencies are defined as time-critical situations in which the delay caused by obtaining a written order may be detrimental to the patient.

I. Emergencies include, but are not limited to, the following:

- a. Cardiac arrest
- b. Cardiac Arrhythmias
- c. Hemorrhage
- d. Impending coma
- e. Impending shock symptoms.
- f. Respiratory distress.
- g. Seizures.
- h. Shock.
- g. Delivery room; during mother / fetal distress.

II. Limited telephone orders to address a specific urgent issue may be accepted when the provider is:

- a. Not present in the hospital or physically caring for a patient in the Emergency Department.
- b. Directly responding to a telephone request from a staff member authorized to accept telephone.

III. Verbal or telephone orders **cannot** be given:

- 1. For cytotoxic agents' investigational drugs or blood transfusions except in an emergency.
- 2. For chemotherapy medications.
- 3. Routinely, in place of daily written orders.
- 4. Epidural boluses - unless authorized Acute Pain Management Service (APMS) Nursing Staff.
- 5. Blood/blood products (verbal orders may occur in theatre or ED under emergency situations e.g. the Massive Transfusion Protocol).
- 6. Patients with documented history of chronic kidney disease or acute renal failure.
- 7. Abortion inducing medications.
- 8. Labor inducing medications

Patient Identification:

This policy requires that all patients to be identified by **two identifiers (Refer to the Identification and Verification Policy available in the document management system).**

Process of Receiving Orders

Verbal and telephone orders from authorized providers will only be accepted if the patient's identification is confirmed using two identifiers. The order will be entered in Al Shifa system within the shift and with maximum of six hours post order stating that this order was verbally given.

Verbal and telephone orders must be accepted and processed by designated healthcare professionals: doctors, nurses, pharmacists, respiratory therapists, dietitians, physical therapists, occupational therapists, speech therapists, and perfusionists.

1. The healthcare professional receiving the verbal order should verify the identity of the physician to ensure that the order is coming from an authorized source.
2. The healthcare professional, upon receiving the verbal order, reads back the details of the order to the physician to confirm that it has been accurately understood.
3. Both parties should pronounce the order clearly.
4. For medication order, the prescriber should spell the name of any unfamiliar drugs.
5. The order must be written within the shift and with maximum 6 hours post verbal order.

Roles and Responsibility:

a) Nurse:

- 1) The nurse is responsible to ensure there is a valid, complete medication/treatment order prior to the administration of prescriptive medications or the implementation of a medical intervention/treatment.
- 2) The nurse has the right and responsibility to confirm orders when there is a question of authenticity or accuracy of the orders. . Read-back the order to the prescriber for verification even if the receive is confident they heard the order correctly.
- 3) The nurse is responsible to recognize the appropriateness of the order with respect to the plan of care, and for implementing the order, or obtaining clarification from the prescriber.
- 4) Orders must be complete enough so that no further medical judgment is needed when the order is implemented.
- 5) Nurses must adhere to established protocols and policies when handling verbal orders.

b) Physician:

1. Physicians should ensure clear and complete communication when providing verbal orders. It is essential to convey the details accurately and comprehensively to the healthcare professionals receiving the orders.
2. The physician's responsibility to ensure that the orders are acknowledged by the appropriate healthcare professionals.
3. Physicians should use verbal orders judiciously and in situations where immediate action is required and they are unable to use written orders.
4. Verbal orders should be given with the patient's best interest in mind, ensuring that the orders contribute to the patient's well-being and recovery.
5. Physicians must adhere to the established policies when handling verbal orders.

c) Allied health:

1. Allied healthcare professionals are responsible for accurately receiving and verifying verbal orders from physicians. This involves ensuring that the orders are clear, complete, and consistent with the patient's needs and medical history.
2. Allied healthcare professionals must promptly and accurately document the orders in the patient's medical record.
3. If there is any ambiguity or uncertainty regarding verbal orders, allied healthcare professionals are responsible for seeking clarification from the physician.
4. Allied healthcare professionals must adhere to established protocols and policies when handling verbal orders.
5. Ensure that the patient's safety and well-being are protected, and the orders align with the best interests of the patient.
6. Clear and effective communication among allied healthcare professionals is crucial in conveying and implementing verbal orders. They must communicate with one another to ensure that the orders are accurately understood and carried out.

Training

1. Directors, heads, leaders and in-charges are responsible to train their employees on this policy on regular basis and ensure awareness sessions are conducted and recorded.
2. Hospital employees: are responsible to familiarize themselves with policy and seek guide from their leadership team for further support.
3. All training records (soft copies) to be submitted to the Quality & Patient Safety for monitoring.

Measuring compliance:

- 1) Directors, heads, leaders, and in-charges are responsible to audit compliance to the policy on a regular basis.
- 2) Audit reports to be submitted to the Quality & Patient Safety for monitoring.
- 3) Report all incidents related to breaching the policy and any related events.

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References:

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